NIHR | National Institute for Health and Care Research

NIHR Research for Patient Benefit Programme

Scheme	Research for Patient Benefit Programme
Funding Opportunity	2025/224 RfPB Researcher-led

SUMMARY INFORMATION

Application Title	Enhancing inclusion and care of victim-survivors of lifetime sexual harm in the emergency department
Contracting Organisation	UNIVERSITY HOSPITALS COVENTRY AND WARWICKSHIRE NHS TRUST
Contracting Organisation Department	Research & Development
Start Date	01/07/2026
End Date	30/06/2028
Duration (months)	24
Total Cost to NIHR	£207,964.33
NHS Support Costs	£3,316.50
NHS Excess Treatment Costs	£0.00
Non-NHS Excess Treatment Costs	
Has this application been previously submitted to this or any other funding body?	No

LEAD APPLICANT & RESEARCH TEAM

Lead Applicant

Full Name	Professor Lorna O'Doherty
Organisation	Coventry University
Department	Healthcare & Community Transformation
Proposed Role	LOD will undertake strategic leadership, project governance and reporting; engage stakeholders; provide project oversight and support co-applicants and staff; lead on developing programme theory; contribute to data analysis, interpretation, film production, intervention design and knowledge sharing.
ORCID	0000-0003-0816-9321
% FTE Commitment	10.00%

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Co-Applicants

Full Name	Dr Caroline Leech
Organisation	UNIVERSITY HOSPITALS COVENTRY AND WARWICKSHIRE NHS TRUST
Department	Emergency Department
Proposed Role	CL will enable and guide recruitment of NHS staff stakeholders, provide insight into interpretation of findings from a clinical perspective leading to co design of the intervention, and advise and participate in dissemination to healthcare professionals.
ORCID	0000-0002-5301-9798
% FTE Commitment	5.00%

Full Name	Dr Eleanor Lutman-White
Organisation	Coventry University
Department	Centre for Healthcare and Communities
Proposed Role	ELW will lead the qualitative data collection and analysis, oversee all elements of the experience based co-design approach, lead recruitment of survivor participants, support the work of the PPI partner, and oversee production of creative outputs and write up of the project.
ORCID	0000-0003-0219-7567
% FTE Commitment	40.00%

Full Name	Ms Nikki Holliday
Organisation	Coventry University
Department	Centre for Healthcare and Communities
Proposed Role	NH will be co-design technical lead for the project, designing and delivering the co-design workshops/events, and will advise on intervention development.
ORCID	0000-0001-7837-3817
% FTE Commitment	5.00%

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Full Name	Ms Arti Naithani
Organisation	RESHAPERS COMMUNITY INTEREST COMPANY
Department	Survivors Voices
Proposed Role	AN will lead PPI input by engaging the involvement of others and co-directing the research, shaping methodological decisions, developing participant-facing documents/information, supporting survivors at in-person events, interpretation of data and writing, supported by a Team from Survivors' Voices.
ORCID	0009-0004-8693-9074
% FTE Commitment	13.00%

APPLICATION DETAILS

Scientific Abstract

Research question

What are the components of an intervention to embed trauma-informed practices responsive to the needs of survivors of sexual violence and abuse in emergency departments (EDs)?

Background

Sexual violence has lifelong impacts on survivors' physical and mental health, is associated with significant health inequalities and very costly to society. Survivors often avoid healthcare (e.g. cancer screening) due to prior negative encounters with services and fears about re-traumatisation. EDs are a vital entry point to the NHS, but the setting (e.g. urgency of care, fast-paced environment) creates significant access challenges for trauma survivors. Among the 25 million annual ED attendances in England, over 4m are estimated to be survivors of lifetime sexual violence. Current ED practices may aggravate distress, self-discharge, and poor adherence to advice. Staff report feeling underprepared to support survivors. Trauma-informed care can transform the healthcare encounter by prioritising safety, trust, choice, collaboration, and empowerment. However, there is limited evidence on introducing these practices into EDs.

Aims and objectives

This study aims to co-design a complex, trauma-informed intervention to improve ED care for survivors of sexual trauma.

Objectives:

1. Examine lived experiences of survivors accessing ED care and staff who provide the care and prioritise what needs to change at the patient-clinician interface, and for health professionals, the environment and organisation.
2. Identify 'functions' of the planned intervention
3. Co-design intervention components
4. Develop an underpinning theory of change.

Methods

Integrating Experience-Based Co-Design with a behaviour change framework, survivors and staff will come together to co-design an intervention. Arts-based methods will underpin discovery, co-produced content and knowledge transfer. Partner organisation, Survivors Voices, will lead on work with people and communities, provide peer support and expertly facilitate co-design groups to engage survivors.

Phase 1 M1-16 Identify the need for change using focus groups with NHS staff (n=20), and narrative interviews with survivors (n=30) recruited in health and community. We will gather narratives and 'touchpoints' of ED care to form the basis of arts-based outputs and prioritise targets for change.
Phase 2 M17-18 Define intervention functions using a co-design event with a mixed subset of 20 participants.
Phase 3 M19-24 Co-develop prototypes for the intervention in 3 mixed sub-groups with 15 participants, such as skills training, resources tailored to the ED, local agreed referral pathways.

Impact and mobilisation

Findings to be shared widely in accessible formats with community and survivor organisations and key professional bodies e.g. RCEM and RCN. This work will provide a foundation for quality improvement, healthcare policy, ED service design, NHS efficiencies and better health and healthcare for survivors. We will refine a theory of change and prepare for the next stage, to address uncertainties and demonstrate promise in a feasibility study.

Inclusion
Boosting our work with survivors and communities, an inclusion strategy will expand reach to underserved communities at the intersection of a high prevalence of sexual violence, poor health and marginalisation. Actions include geographically and demographically diverse NHS sites, local champions, and bespoke support for participation.

Plain English Summary
What's this study about?
This study is about improving emergency department care for people who have experienced sexual violence and abuse.

Why it matters
Sexual violence causes deep and lasting harm to survivors, their families, and communities. More than 10 million people in Britain have experienced sexual violence as children or adults. Sexual violence affects physical, sexual, and mental health. But visiting the GP, having screening tests or attending hospital can feel uncomfortable, even distressing for survivors. Survivors may worry about not having a say in what happens to them or their bodies, being dismissed or blamed for their difficulties. They may re-live feelings or memories connected with abuse. Many survivors avoid healthcare altogether which can make health worse.

Emergency departments are especially difficult for survivors – the care is urgent and unavoidable. The environment is busy, noisy, and very clinical. Emergency departments could be more survivor-centred – like offering greater choice, showing care in asking about abuse, or for consent before examinations or treatment, and taking steps to help a person feel safe. These changes could improve fairness and accessibility, survivors' health and lower NHS costs.

What this study will do?
We will plan a new approach to care for survivors. We will do this using a type of research called 'Experience-Based Co-Design'. Survivors will be invited to share about times they went to the emergency department. NHS staff will tell about caring for survivors as patients. Using what survivors and staff tell us, we will work with survivor-artists to create powerful outputs, including a short film. Next, survivors and staff will work together to agree goals for the new approach for emergency departments. For example, a goal might be to improve NHS staff understanding about the health impacts of sexual violence or create a safer environment. Finally, smaller groups of participants will plan how to make changes, like showing staff how to communicate concerns about abuse.
Survivors and community leaders were involved in planning the research to date, especially on including survivors from a range of communities, safely and fairly. They advised that arts-based methods are an excellent way to share about research and that health services can be places of healing over distress. It is vital survivors continue to co-lead this work.

What will we do with the results?
We will set out a new approach to emergency care that allows survivors get the healthcare they need. We will plan a follow-on study, looking at whether the approach could lead to better outcomes for survivors, staff and the NHS. We will explore if it might be possible and sensible to roll-out to emergency departments nationwide in the future. Creative outputs will explain what we did and learnt.

Changes from Previous Stage
We thank the committee for their review and constructive comments. Please find below responses to the points raised.

1. The rationale of proposed study was unclear...clarify how arts-based and creative outputs would feed into intervention development and what final intervention might be.

We included stronger rationale based on gap in evidence for transforming EDs from places of distress

into environments of healing and empowerment for survivors, and a policy impetus to reshape EDs for better sexual violence and mental health care in the future.

Arts-based methods are central to the participatory approach needed to achieve the aims. Endorsed by PPI, creative outputs (e.g. 5-min film reflecting stakeholders' needs-in-context) register the collective effort of staff and survivors. They feed into intervention development by 1) stimulating co-design work 2) being integrated into the intervention (e.g. to impart skills, educate, stimulate action) and 3) sharing key messages in ways that engage and are accessible.

The final intervention is based on priorities identified, existing theory/evidence and the co-designed functions/features. It will include elements to target change at the patient-clinician interface, healthcare professionals' practice, environment and systems e.g. skills building, patient resources, referral pathways (see Figure 1, Detailed Research Plan).

2. Concerns raised on aspects of research plan and methodology.

(i) Provide more information on how participants would be recruited, taking into account safeguarding, particularly if participants would be self-identifying as survivors

To ensure safety (see Box 3, main application) we will model trauma-informed principles in the research process and prioritise participants' physical and emotional safety and consider needs of stakeholders, recipients of project outputs and the team. The PPI partner (Survivors Voices) has 20 years' experience of working for 'a safer society with support for survivors that is survivor-sensitive, empowering and services that are trauma-responsive'. The inclusion lead (Little Ro) brings deep expertise in safely engaging survivors from marginalised settings (e.g. with Black and people of colour communities).
Key safeguards include:
• Careful sampling/recruitment, check individual readiness, peer support
• Engage NHS site safeguarding teams in set-up
• Up-to-date resources, safe follow-up
• Responsiveness to risk/signs of domestic abuse, mental health concerns, mandatory reporting requirements
• Maximise participant choice and agency
• Clear boundaries for group work and use survivor-led facilitation
• Responsive to staff needs e.g. workplace pressures, abuse, personal trauma.

(ii) Outline number/type of sites involved

There will be 2 socio-economically differentiated sites: University Hospitals Coventry & Warwickshire NHS Trust (UHCW) and South Warwickshire University NHS Foundation Trust (SWFT). Both sites are Type 1 EDs - one is a major trauma centre (UHCW) and one is a local emergency hospital (SWFT).

(iii) Clarify claim the intervention would be multi-level

Recognising that outcomes are shaped by complex, interacting factors across a complex healthcare system, and consistent with existing research on transformative care for abuse, the intervention would target change at the levels of patient experience, healthcare professionals' practice, environment and systems.

(iv) Address concerns about feasibility and sustainability of the proposed approach, given staff turnover and environment in EDs

Feasibility longer-term will be established in future testing and exploring implementation and scale-up in the NHS. High staff turnover, pace and pressure of the ED also present challenges for the early phase research. All engagements with staff will be done with care recognising unique challenges for this

professional group and physical/psychological trauma as core to the work of the ED. The current research seeks 108 hours of staff time (activity/travel) split across 2 sites. Senior clinical champions will lead recruitment with CL, who brings extensive experience in engaging staff as UHCW's lead for research in the ED. Regular check-ins will be scheduled to pre-empt challenges. CL will identify barriers to engagement early, offer flexible participation in co-design, provide incentives (vouchers/NHS research costs for time/travel), highlight benefits from the intervention (e.g. increased confidence, reduced stress, and enhanced trauma awareness) and creatively celebrate progress.

(v) Time commitment for some co-apps considered insufficient...should be reviewed and justified or amended keeping in mind the funding limit

The budget attempts to balance having the right time/expertise with sufficient resources for safe, successful delivery and outputs. One change was to increase the PPI partner's (AN) time from 5-13% for more co-design and knowledge exchange. They bring advocacy, peer support, access to networks for recruitment and dissemination–all vital for success. CL (5%) will enable staff recruitment, championing the project inside their usual workplace. We will streamline CL's time to optimise focus on the final intervention co-design. ELW (40%) has worked with lead applicant for 5 yrs within an established group, affording efficiencies in project management, starting immediately and progressing at pace. ELW's co-location at UHCW with CL allows efficient study set-up and rapid focus groups. NH (5%), expert in service design, will draw on an extensive design toolkit for rapid prototyping. The lead applicant (10%) has managed large (NIHR) studies to time/budget (e.g. through pandemic) and will drive project efficiencies, governance and support ELW in the day-to day running.

(vi) Committee requested the PES be rewritten to improve accessibility

We have worked extensively with 3 survivor-experts and service leaders to rewrite the summary, eliminating complex language and simplifying sentences.

Working with People and Communities Summary

Our 'Working with People and Communities' summary makes reference to 3 strategies that underpin the project: (1) Approach to safety; (2) Involvement of survivors as equal partners in the research; and (3) Inclusive research.

(1) This research relies on the participation of survivors of sexual trauma to advance its goal of enhancing the capability and commitment of NHS emergency departments (EDs) to provide trauma- and abuse-informed care to those who access its services. Abuse erodes safety and trust; thus, a core principle of our work is to enable people and communities affected by abuse to feel physically and emotionally safe over the course of the research. We know that historically research has been extractive especially in racialised and marginalised populations, and it can lead to re-traumatisation in the context of gender-based violence and other traumas (e.g. racial discrimination and abuse). Therefore, every opportunity must be taken to avoid causing harm to the people and communities we work with – as participants, NHS personnel, advisors and knowledge users. With our goal to develop trauma-informed practice in the ED, it is vital we model best practice in safety, as a pillar of this practice. Our approach to safety (see Box 3 in Detailed Research Plan) sets out measures for ensuring the welfare and wellbeing of all involved.

(2) This project is rooted in participatory action research. Working with people and communities affected by the research is fully embedded across all stages—from design of the research to dissemination—to ensure the research is safe, relevant, responsive and meaningful to survivors. Survivors' Voices (represented by co-applicant AN) is a national, survivor-led organisation with expertise in enabling the involvement of survivors in research through the best practice frameworks it has developed. The researchers have previously collaborated with the team at Survivors Voices who, for the current project, proposed a programme of work that will train our staff, coordinate lived experience input, shape and

support recruitment of survivors, intervention development, creative activities/outputs, data analysis, knowledge-sharing, and crucially, provide peer support to any participant with lived experience who would value it (see Working in partnership with those affected by your research, in Detailed Research Plan).

(3) Survivors of abuse are not a homogeneous group; and their needs are shaped by intersecting identities such as class, ethnicity, gender, and disability. Survivors from marginalised groups face disproportionate barriers to care and support. Abuse can aggravate existing vulnerabilities, social isolation, inequalities and put people at risk of further violent victimisation. Previous research shows that people with mental health issues, insecure housing, cultural minorities, and migrant women are among the most excluded from early care. A priority of our work is ensuring the co-designed ED intervention is fit for purpose - that it responds to all survivors and supports ED staff to meet the needs of the most underserved. Achieving this demands that we represent a wide range of survivor voices. We will engage an Inclusion team (led by a survivor-activist and leader) to ensure fair distribution of power, voice and knowledge, using the funding to monitor and tackle barriers to participation of survivors of diverse and marginalised communities (see Inclusive research, in Detailed Research Plan).

SUMMARY BUDGET

Justification of Costs
Prices below are at the NIHR funded rate.

Staff
Applicants and co-applicants: Lorna O'Doherty (10%FTE) as Chief Investigator to oversee the project. Co-applicant Eleanor Lutman-White (40%FTE) to lead day-to-day running of the project, gain approvals, lead participant recruitment, undertake qualitative data collection, data analysis, write-up and dissemination. Co-applicant Caroline Leech (5%FTE) will provide clinical oversight and lead NHS staff recruitment. Co-applicant Nikki Holliday (5%FTE) is the co-design lead for the project and will advise on intervention development. Co- applicant Arti Naithani (13%FTE) from our PPI Partner Survivors' Voices will ensure PPI is embedded throughout the co-development, delivery and reporting of the project.

Research staff: To support inclusive research practices, inclusion lead Roz Etwaria (Little Ro) will commit 43 hours to the project for inclusive research training and leading inclusive recruitment. Additional interviewers (20 hours) are also requested to accommodate requests by participants to be interviewed by someone who reflects aspects of their identity that are not reflected in the main research team and a local diversity 'champion' (40 hours) is requested to support inclusive research locally.

Other staff: Research Management (£6,380.00) has been requested for contractual agreement, IP and financial management, support and expedite research coordination as lead NHS Trust, and ensure appropriate research and clinical governance, monitoring, and oversight of the study. NHS study coordinator (£6,380.00) will support with contracting, financial and monitoring reports to funder or project leads, attending meetings, liaison with UHCW project team as required and external team.

Travel, subsistence, conferences
£216 is requested for researcher travel to engage with local organisations who will support participant recruitment and travel to staff focus groups at NHS sites.
PPI co-applicant Naithani and the Survivors' Voices team will deliver training to the research team, co-facilitate the Phase 1 workshops and the Phase 1 main co-design event, requiring travel (£900), accommodation at standard rates (£1200) and subsistence (£300).
Funds are requested for attendance at the Royal College of Emergency Medicine annual conference (£1,128). This covers conference fees, travel and accommodation at standard rates.

Knowledge mobilisation, dissemination and impact
£10,260 is requested for creation of a short film by a survivor-artist company, Traumascapes, which will be used to inform co-design work, act as intervention content and for sharing research findings.
£4,000 is requested to transfer the prototypes into a high fidelity suite of resources/toolkit ready for evaluation.

Research Inclusion
We request costs (£8,513.73) above the usual tier limit to ensure our research is designed, recruited, delivered, and mobilised inclusively. Research staff to support research inclusion included above in Staff. Travel costs for the inclusion lead to attend Phase 2 workshops are requested (£320).
£920 for translation of participant recruitment posters and other participant-facing documentation into 5 languages.
£1,670.40 for 20 hours of interpreters across the project for translation as required including consenting, interviews, Phase 1 workshop, Phase 2 main co-design event, Phase 3 co-design sub-groups, peer support.
£480 for travel costs to enable carers or supportive persons to attend in-person events with survivors to facilitate their attendance.
£336 for survivor participant care costs (e.g. childcare) to facilitate participation in the research.
We aim to conduct the majority of survivor interviews remotely. However, £400 is requested for researcher travel for interviews for participants who do not have a phone or internet, or prefer to meet in person for other inclusion reasons.

Working with peoples and communities
Included under other headings.

Other direct costs
Transcription of 30 interviews and 4 focus groups (36 hours; £2,304).
Incentives for participants for all three phases of the research: 30 interviews at £25 each (£600), 15 participants in Phase 1 workshop (£600), 10 participants in Phase 2 main co-design event (£400) and 6 participants in Phase 3 co-design sub-groups (£240). Survivor participant travel reimbursement is also requested (£1,400).
£4,104 has been requested to enable NHS staff participation from 2 NHS sites, including £1,000 for focus group participant voucher (10 staff x 2 focus groups), £1,552 for Phase 1 Workshop (10 staff half a day + travel), £1,552 for Phase 2 co-design event (10 staff half a day + travel).
£3,030 is requested for venue hire, reasonable refreshments and minor costs for printing and co-design materials for project associated with the focus groups, Phase 1 workshops, Phase 2 main co-design events.

HEI Indirect costs
Coventry’s indirect costs are requested at 80% FEC, as per NIHR guidance.

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ACKNOWLEDGEMENT AND CONFLICTS

I have read and understood the terms of applying as Lead Applicant and accept this role	✓

Potential Conflicts	None identified

****Background and rationale****

What is the health or social care problem being addressed?

"*Healthcare is frequently inaccessible to survivors of sexual violence, it takes strength to allow someone to help you when you cannot bear to be touched, particularly within the emergency department where survivors need immediate medical attention and there is no time to process what is happening*." (Public & Patient Involvement member)

Sexual violence and abuse (*henceforth sexual violence or harm*) is a violation of human rights and public health problem of epidemic proportions. It is the leading contributor to post-traumatic stress disorder (PTSD) worldwide (1). The cost (to the NHS, other public services and society) in England and Wales is estimated at £30 billion per year (2,3). Sexual violence is recognised as both a cause and consequence of gender (4) and health inequalities (3,5), but affects all socio-economic, gender, cultural, sexual orientation, and age groups. Survivors (preferred term in the communities we work with) are at risk of lifelong consequences for mental and physical wellbeing and have poor social, education and economic outcomes. One in 4 women and 1 in 20 men have lifetime exposure (4) and the problem shows little sign of abating (5). Stigma, shame and silencing of survivors produce significant distress and harm. Survivors experience a high rate of physical and mental health problems over the life course including chronic pain, fibromyalgia, gastrointestinal disorders, gynaecological problems, distress, self-harm, suicidality, PTSD, anxiety, depression, eating disorders, substance misuse and prescription drug use (6–10), producing high healthcare needs (6,11). However, survivors of sexual harms:

- are rarely asked about experiences of sexual violence in healthcare, thus critical opportunities for safe identification, immediate support and referral are missed (12)
- are at risk of re-traumatisation in healthcare settings (13,14) (see Box 1). Our prior research found that survivors accessing voluntary sector organisations and Sexual Assault Referral Centres (SARCs) experienced high standards of sexual violence care in these settings; however, 1 in 6 experienced NHS services (including general practice, mental health services) as harmful when they sought help after abuse (6).
- delay or avoid cancer and other healthcare screening (15) and help-seeking for *any* symptoms due to previous adverse experiences in the NHS and fears about how they may be treated (16–18)
- have cumulative health inequalities (7,19–21), aggravating risk for re-victimisation
- see a compounding of these effects when they are also from marginalised groups such as gender minorities, have chronic mental health problems including complex PTSD, are poor or from migrant and other culturally diverse communities (6,22,23).

Box 1 Signs of re-traumatisation that may arise during healthcare encounters include dissociation, hostility, disengagement and self-discharge. Precipitating factors include uniforms, procedures (e.g. catheterisation, removal of clothing), clinical environments or equipment, lack of empathy, and poor consent processes.

Most survivors do not access timely specialised care like that offered by SARCs (5,6) and cumulative trauma is common in this population e.g. 75% of adults seeking help at SARCs had 4+ adverse childhood experiences and 60% were sexually abused as children (6). Health services generally are poorly equipped to meet the complex needs

of survivors: they may lack staff trained in abuse and violence care; have limited resources; not consider violence a health problem; be reluctant to talk about it; and not know how to help (24). Survivors have highlighted a range of NHS services that require urgent attention to address how they provide healthcare and respond to the needs of survivors of sexual violence (James Lind Alliance; 23), including emergency departments (EDs). EDs are a vital entry point to NHS care and may be a first point of contact for sexual violence survivors. Among the 25 million ED attendances in England each year (26), 4 million are estimated to be patients with a lifetime exposure to sexual harms (27). This sub-population is made up of those seeking ED healthcare in the immediate aftermath of sexual assault or rape; in response to needs arising from trauma in the longer-term (e.g. self-harm, suicidality); and for other needs (unrelated to the abuse). Irrespective of the reason for the visit, the often urgent and unavoidable nature of ED care together with the noisy, fast-paced and highly clinical environment directly challenges the delivery of trauma-informed care (see Box 2). NHS staff can feel ill-prepared or lack motivation or confidence to provide care for survivors (28,29). The ED may therefore lead to avoidable distress and re-traumatisation, self-discharge, inefficiencies in healthcare delivery and poor outcomes post-discharge, including lack of uptake of referrals and onward care and avoidance of future healthcare (13,14,16,17,30).

Box 2 Trauma-informed care is grounded in holistic understanding of how exposure to trauma affects neurological, biological, psychological and social development. The model recognises how trauma affects individuals, organisations and communities and the signs of trauma in service users and providers. It emphasises agency and choice; adopting a consistent, non-judgemental approach; and creating a safe environment for all patients and staff and actively resisting re-traumatisation (30).

What is the gap the research fills and what existing research can inform it?

Trauma-informed care is increasingly adopted as an approach to clinical care in both primary and secondary care, including emergency medicine (31). US studies have shown cost-effectiveness with clinical benefits to patients and job satisfaction benefits to staff (32). However, despite the immense burden of trauma seen in the ED and the benefits for patients and clinicians, trauma-informed care remains an emerging field in emergency medicine. In the UK, there are barriers to integrating these practices in EDs e.g. one priority for emergency medicine, which aligns with the proposed work, is to optimise the delivery of mental healthcare but this may be at odds with another to reduce wait times (33,34). Yet enhanced provision of care could help alleviate problems that underpin wait times like efficient discharge, referral and reduced readmission (35). More evidence is needed to understand needs, challenges and opportunities for ED clinicians and patients (36). Currently, no UK study integrates the perspectives of staff, patients, decision-makers and marginalised communities. Another gap concerns *intervention* research for trauma-sensitive care of sexual violence survivors in EDs. A recent systematic review synthesised findings from 10 studies of trauma-informed care ED interventions (32). Just two targeted the needs of sexual violence survivors; they showed improved clinician knowledge, care and collaboration with community services but lacked patient outcome data, focused on *recent* sexual violence only, and were outside the UK (32). Most of the intervention research in this area concerns screening and care for domestic abuse. A Cochrane

Review by the lead applicant reported that whilst ED screening for domestic abuse increased identification/disclosure, there was a lack of evidence for enhanced referral practices and health outcomes for survivors (37). The WHO has consistently identified that certain conditions must be satisfied for effective care for sexual/gender-based violence, including confidentiality, trained staff, clear referral pathways and policies (24). Whilst a trauma-informed ED is vital for people living with current/recent abuse, it may also reduce distress, prevent re-traumatisation and promote resilience among people with lifetime exposure. Existing research on transforming health settings to be responsive to people's needs after abuse can inform novel interventions, including that implementing this approach requires a cultural shift within healthcare organisations (30). Interventions need to be complex and multi-modal, strengthening health system leadership and governance as well as a focus on service delivery and providers' capacity to respond to trauma and abuse (38).

Why is this research important and how does it address the aims of RfPB?

With an estimated 4 million survivors of sexual violence using UK EDs each year (26,27), a lack of trauma informed care in EDs means significant missed opportunities to effectively identify/refer patients as appropriate to specialist or community care and deliver healthcare in ways that makes a tangible difference to people's lives, prevents further harm, supports recovery, addresses inequalities and improves life chances. Royal College of Emergency Medicine (RCEM) guidance on management of sexual violence in the ED is a decade old and extremely limited. Further, mental health needs make up 5% of ED visits (39), with a recent meta-analysis suggesting 30% of women mental health service users are sexual violence survivors (40). Addressing mental health need is a priority for emergency medicine (34), but existing RCEM guidelines do not address trauma-informed care (41). The current research is a vital opportunity to co-produce evidence on *how* to normalise trauma-informed, survivor-centred practices and supports the Government's goal for a reimagined NHS that tackles inequalities in access and outcomes and avoidable harms (42). It can inform the codesign of dedicated mental health EDs outlined in the NHS 10 Year Plan (42). Building on our extensive clinical and research experience in health and emergency settings, gender-based violence and partnerships with survivors, artists and advocates (6,43), this project will develop a theory-driven, complex intervention to embed best practice and transform how care is delivered to survivors in the ED, irrespective of where a patient is in their recovery (e.g. has or has not disclosed the abuse), such that every ED contact counts in shifting from distress to healing. Recognising the need for an innovative yet pragmatic study, intervention development will be participatory, arts-based and promote a re-scrutinising of the ED space and experiences of those occupying it, capturing priority voices and attending to uncertainties, barriers to implementation, and feasibility from the outset (44–46). The project will result in a coproduced theory-driven intervention to drive service improvement. We will go on to determine if the proposed intervention can demonstrate promise in a feasibility study. The intervention will realise patient benefit should a definitive trial show effectiveness by reducing adverse emotional experiences/re-traumatisation; increasing satisfaction with the ED encounter, disclosure and onward referral to specialist sexual/domestic violence services. Rather than prompting fear, distrust and distress, this model of care could promote understanding, safety and dignity, translating longer-term into reduced healthcare avoidance and better self-care, less risk, better health, functioning and healing, and reduced health inequalities, ED visits and costs for the NHS (2,3,47).

****Aims and objectives****

This project aims to co-design a multi-level intervention to transform care for survivors of sexual trauma in emergency departments (EDs). The complex intervention would target change at the levels of patient care, healthcare professionals' practice, environment, and systems. The research objectives are: (1) increase understanding of survivors' experiences of ED care and the experiences of ED staff who provide it and prioritise what needs to change; (2) identify the 'functions' of the intervention to improve quality of care for survivors attending EDs; (3) co-design intervention components (e.g. interactive training; clinical skills development; referral pathways) with NHS staff stakeholders and service users, and (4) develop a theory of change to support the co-designed intervention.

****Methodology / Plan****

1. **Project design and methods**

We will follow the MRC complex intervention framework to ensure we design a testable, reproducible intervention to normalise trauma-competent practices in the ED (48). We will employ a theory-driven approach, integrating Experience-Based Co-Design (EBCD) (49,50), COM-B and the Behaviour Change Wheel (BCW) (51,52), to maximise the fit between the problem and proposed solution (see Appendix 2). A participatory action research approach will bring survivors of sexual violence, NHS clinicians and other stakeholders together to co-design an intervention, ensuring it is relevant and implementable under the unique conditions of this setting (53). EBCD gathers lived experience to identify challenges and opportunities for improvement and fosters shared ownership of solutions through co-design process (37). In this project, arts-based methods underpin discovery and co-production; are proposed for use in intervention content and will enable knowledge transfer into professional, survivor and diverse communities. The emancipatory character of arts-based research reduces barriers to participation and inclusion (54) and removes obstacles to collaboration between patients, NHS staff and researchers by rebalancing power relationships (55). Arts approaches are 'disruptive' and promote a 're-scrutinising' of the world (44) e.g. visual art/theatre and film to allow audiences to vicariously experience survivors' encounters with ED care, and re-think assumptions about patients (e.g. survivors as 'uncooperative') and 'valid' knowledge (56). Arts-based approaches also open avenues to effectively translate knowledge (57). The project aims to develop innovative modalities for training and arts-based approaches emulating patient experience will foster learner engagement (58). There will be 3 phases aligned to EBCD and BCW (see Appendix 2), mapping experience-gathering stages of EBCD to BCW Stage 1 (understanding the problem); using the main co-design phase in EBCD to identify intervention possibilities (BCW Stage 2); and aligning the co-design sub-groups stage of EBCD to BCW Stage 3 (identify content and implementation options).

Eligible participants

Group 1:

- Individuals of any gender ≥18 years
- Experienced sexual violence at any age (self-reported)
- Experience of NHS emergency care within last 5 years
- Capacity to consent

Individuals will be excluded if safety or wellbeing are compromised by participating e.g. severe mental health problems or current domestic abuse (see Box 3).
Group 2: ED staff (clinical/non-clinical) and other key NHS staff stakeholders (e.g. leads for strategy, improvement, safeguarding, drug and alcohol teams, training) at either the University Hospitals Coventry & Warwickshire NHS Trust (UHCW) or South Warwickshire University NHS Foundation Trust (SWFT) site.

Recruitment

To ensure the breadth of inclusion and fulfil recruitment targets, survivors will be invited to participate via a range of organisations where we have trusted links: specialist/community voluntary sector organisations (Foleshill Women's Centre; Coventry Haven Women's Aid; Safeline; CRASAC, Little Ro - see Appendix 3 Letters of Support). Our PPI partner, Survivors' Voices, will support recruitment via social media and a mailing list of 2500 survivors; this will be supplemented by posters/leaflets displayed in waiting rooms at the participating NHS sites. We will engage Coventry and Warwickshire Partnership Trust, mental health service provider for the region. To reach potential participants we will share multimedia information about the study which will invite potential participants to contact the project team if they are interested to hear more. When potential participants self-refer, we will provide information, answer any questions, and review with the person the eligibility criteria and ways for the person to be involved. If they agree, we will collect basic demographic data to aid sampling, before planning for participation in Phase 1. Our recruitment strategy was agreed with our PPI partner/team and inclusion lead (as part of an Inclusive research adjunct to the proposal, see section 5) who will inform coproduction of materials and monitor recruitment for diversity and safety. NHS staff will be recruited from UHCW and SWFT, 2 Type 1 EDs (one major trauma centre and one local emergency hospital) which are a mix of size, urban/rural, diverse patient populations. Study information will be circulated to ED staff via department managers and research leads. Key NHS staff stakeholders (e.g. leads for strategy, improvement, safeguarding) will be reached via Trust Heads of R&D. Staff can express interest by contacting the team. Our clinician co-applicant (CL) will shape staff recruitment materials, with input from PPI/inclusion teams to acknowledge that staff may also have histories of different forms of trauma.

Proposed sample size and sampling strategy

We aim to recruit 30 survivors and 20 NHS staff stakeholders, based on the level of 'information power' we anticipate reaching (59). In line with our previous qualitative research with survivors including in health contexts (6,60,61) we expect that those who engage in the study will contribute large quantities of rich data, and we are taking an exploratory approach with minimal pre-existing evidence to guide us. Therefore, we have chosen modest sample sizes which, taken together (n=50), will result in a substantial dataset. Guided by our approach to working with people and communities, a safety standard (Box 3) and inclusion lead (see Inclusive research), we will use maximum variation sampling for a participant group diverse in age, disability, ethnicity, language, gender, sex, sexual identity, health conditions and socio-economic situation with varied exposures to sexual violence (e.g. childhood, adulthood exposure, domestic abuse context) and experiences of NHS care. We will purposively sample ED staff including clinical staff and non-clinical stakeholders (see Appendix 2).

Box 3 Approach to safety

The physical and emotional safety of participants from the point of contacting people about the research right through to sharing about findings from the project is a primary concern. Our research process mirrors the trauma-informed care we are advocating for the ED and this starts from the assumption that any person—patient, staff, wider stakeholders—could have a trauma history. The researchers, PPI partner and Inclusion Lead are highly skilled in survivor involvement and working together to embed trauma-informed approaches in the research lifecycle, to identify and mitigate the main risks to safety and wellbeing. This includes, and is not limited to, people experiencing distress, feeling uncomfortable about the topic or disempowered during the co-design work; being reminded about or reliving of aspects of the abuse; having expectations about support from the research that are not fulfilled; and risks associated with reactions of partners, family members, communities to the person's participation. We will draw on Survivors Voices' frameworks (62,63), our prior best practice in a longitudinal study with 350 survivors of sexual violence and abuse and related safety protocols (6) alongside the guidance we receive from ethics committees and NIHR toolkits and safeguarding teams at the two sites. Key actions for safety:

- Exceptional care in sampling and recruitment procedures, particularly when applying inclusion criteria, recognising people will be at different points in healing and access to support. Risks and benefits will be considered with each person on a case-by-case basis. It is recommended that the subset of survivors who proceeds to the co-design phases will have previously accessed support or have this in place currently.
- Participants will be invited to provide written consent prior to interviews/focus groups and we will gain consent at the start of each workshop or co-design event.
- Participants will be offered access to at least one hour of peer support (via Survivors' Voices) to discuss their experiences in the research, with onward referral if required.
- Comprehensive, accessible provision of up-to-date, relevant information on local and national support and resources for all participants including safe follow-up by the team.
- Attention to the wider context of people's lives outside the research (see Inclusive research) e.g. people signalling worries about domestic abuse, workplace bullying, child sexual abuse and other forms of abuse, exploitation and discrimination, mental health concerns such as self-harm or suicidality and ensuring research is responsive to these signs (e.g. interviewers making participants aware of mandatory reporting requirements; ensuring confidence of interviewers to safety-plan and refer to domestic violence services and knowing when and how to involve the police).
- Undertaking due diligence around involvement of any interpreters and translation services to ensure no risks to confidentiality especially within smaller communities.
- Embedding choice and agency at every stage such that any participant is free to take breaks, change their mind, conclude their involvement, or withdraw their data.
- Setting clear boundaries prior to group work for what can be safely discussed and gaining participant agreement, including to show respect for NHS staff participants.
- Recognising/responding to the needs of NHS staff, including pressures of the NHS context; negative or traumatic encounters with patients/families in the ED (workplace abuse and violence) and responses of the organisation to such incidents; own experiences of gender-based violence and preserving staff confidentiality and dignity.
- Recognising/responding to the needs of the team (e.g. vicarious trauma) – the PPI partner will provide training and reflective spaces, preparing the team for abuse- and trauma-informed work, topping up as required and supporting at key junctures.
- Finding opportunities to bring hope for a resilient NHS that serves patients and staff, through teamwork, co-creation, empowerment and celebrating successes.

Participatory research activities

Phase 1: Understand lived experiences and identify what needs to/can change

Participant interviews and focus groups

Narrative interviews (64) with 30 survivors will provide a space for recounting 'stories' of healthcare access and experiences, with researchers guiding the conversation to explore experiences and meanings. Complementing survivors' data and accounting for time constraints for staff, we will run 4 focus groups flexibly (onsite at two sites) with 20 NHS staff to examine experiences and perspectives on managing sexual assault disclosures in EDs, responses and care including presenting physical health issues among those with complex mental health. We will encourage story-telling and gather touchpoints, to inform understanding of barriers, facilitators and best practice relevant to trauma-informed EDs. Permission will be sought to audio record interviews and focus groups and we will use flexible co-developed interview/topic guides.

Data analysis

We will use both narrative analysis methods (64) and reflexive thematic analysis for Phase 1 data (65) to facilitate a deep, individual understanding of survivor stories while also identifying patterns across staff and survivor datasets. We will analyse the survivor interviews for story units which will be preserved as core stories (64). This involves integrating and reordering recounted events into a temporally organised whole. Core stories will form a significant resource for subsequent development of creative outputs but will also be analysed thematically alongside staff focus group data. Transcripts and core stories will be added to qualitative analysis software (e.g. NVivo) for coding. The qualitative lead (ELW) will read through the data to aid familiarity. A subset of the data will be coded by 2 members of the team (ELW and LOD). Codes will be discussed to generate initial themes and patterns of meaning in the data. We will draw on the 'following a thread' method (66), which involves analysing what different datasets illustrate about the same topic e.g. when considering barriers to trauma-informed care in the ED, we will compare data from survivors and NHS staff, analysing divergences and similarities to understand this from multiple perspectives. As themes develop, we will analyse each theme within the data from each set of participants and consider its relevance and dimensions across the participants. The evolving analysis will be written up into an analytic narrative with supporting data extracts, and key messages will be generated to inform the creative outputs. Our PPI partner will be involved in the interpretation of data by attending discussions and reviewing themes as the analysis progresses. The potential themes will be reviewed and developed within the team as analysis progresses and more codes and (sub)themes are developed.

Co-production of outputs

The findings will be used to inform a primary arts-based output 'ethnodrama' in the form of a short film that captures one or more scenarios representing survivors' experiences in the ED. Practitioner experiences may be integrated, or feature as a second separate output (e.g. infographic, illustration of findings, poetry). The film output will be co-designed, working closely with our PPI partner and engaging a survivor-artist company (Traumascapes) to direct and produce the film, utilising local clinical training facilities as a set. Our team is experienced at working with survivors

and producers to develop cultural and creative outputs including film and poetry to raise awareness about different forms of abuse and life as a survivor.

Feedback workshops

Separate workshops with subsets of participants (15 survivors; 10 NHS staff; see Appendix 2 for Flow of participants) will stimulate priority setting to inform the intervention development in Phases 2 and 3. This process will involve reflecting on creative outputs and other findings, and prioritising issues using visualisation techniques such as insight mapping and clustering (67). Core priorities and themes will be mapped onto the **C**apability (e.g. no past training in how to identify or refer for abuse), **O**pportunity (e.g. perceives there is not enough time, privacy, or resource), **M**otivation (not confident it will make a difference, or emotionally uncomfortable) and **B**ehaviour (e.g. enacting range of trauma-informed practices in the ED) components of the **COM-B** diagnostic framework (52) to identify what can/needs to change.

Phase 2: Identify intervention possibilities

Phase 2 will bring stakeholders together for the first time, including 10 survivors and 10 staff participants, to participate in-person in the project's main co-design event. Researchers will present results and priorities for change from Phase 1, integrated with theory and existing evidence. Members will work together, using activities such as storyboarding that support collaborative ideas generation, to identify intervention inputs and 'functions' (e.g. modelling, environmental restructuring) including systems categories like policies, protocols, communication and leadership. Co-design activities will be co-facilitated with our PPI partner. The session will use a range of co-design tools by design co-applicant (NH). The PPI partner, clinician co-applicant (CL) and inclusion lead will ensure safe, productive and equitable engagement for everyone.

Phase 3: Identify content and implementation options

Three co-design subgroups (each consisting of 2 survivors and 3 staff) will meet on 2 occasions to develop and recommend components aligned to different levels (e.g. individual patient, health professionals, environment/systems). This will include identifying Behaviour Change Techniques (BCT Taxonomy V1; 53) that align with the chosen intervention functions from Phase 2 to underpin component development. Subgroups will determine how the intervention components should be delivered in practice by applying the APEASE (Affordability, Practicability, Effectiveness, Acceptability, Side-effects, Equity) criteria (52). Co-design activities will include visual methods such as paper prototyping and storyboarding (68,69) and co-design groups will identify how creative outputs could be integrated into intervention components.

Intervention content will be presented using the Template for Intervention Description and Replication (TIDieR) checklist (70). Across 3 phases, a programme theory (71) will be generated and refined collaboratively (e.g. using visual logic representations to help engage all stakeholders). It will describe the key components of the intervention and how it is expected to lead to its effects and under what conditions (i.e. we will identify contextual factors that may influence mechanisms of change). We will theorise pathways to short and long-term outcomes for survivor/service users, NHS and society. Having a programme theory will enhance transferability of the intervention, enable further testing and aid decision-makers' understanding.

Timeline and milestones

Duration: 1/7/2026 to 30/6/2028; ethics submission 3 months pre-kick off (see Appendix 4 Gantt Chart). **M1-16:** Study set-up, ethics approvals, agree safeguarding arrangements locally on sites; Phase 1 recruitment, data collection, analysis, co-produce ethnodrama/film, participant feedback workshops to review findings/outputs, set intervention priorities, apply COM-B, theory/evidence integration. **M17-18:** Phase 2 co-design event to refine priorities and identify intervention functions. **M19-21:** Phase 3 subgroups to identify multi-level components of intervention. **M22-24:** Refine theory of change, present intervention to stakeholders; final report, funding proposal for a feasibility study and dissemination activities.

2. Working in partnership with those affected by your research

Working in partnership with people and communities is at the heart of participatory action research. Survivors Voices is a survivor-led provider of state-of-the-art services for enhancing engagement of survivors in research, practice and policy. They are our nominated PPI partner, represented by co-applicant AN. Previously Survivors Voices evaluated our own team's approach to working with survivors in NIHR-funded research as "the benchmark" for the field (72). Now, we will go further than ever to embed PPI in every dimension from design to dissemination, ensuring our research is trauma-informed, relevant and centres survivors. The partnership will optimise the theoretical and pragmatic fit between the codesigned ED intervention, and the needs of the populations served. We will be advised on sampling and recruitment procedures (e.g. including and excluding with care), retention of survivors, creating a safe space that encourages participants to express their views freely but with agreed boundaries on what may be shared/discussed, see Box 3); advising on language use; contributing to interpretation of findings in Phase 1; informing creative outputs; and co-facilitating Phase 1 workshops, Phase 2 co-design event and Phase 3 subgroups. The PPI partner will be the primary provider of peer support through a dedicated Peer Support Lead, one of several mechanisms used for harm reduction. Supervision for the Survivors' Voices team is offered by Jane Chevous (director). Survivors Voices is an industry exemplar, trusted by academia, charities, survivors and supporters to involve survivors in research that concerns them. Embedding Survivors' Voices in the leadership of the project will ensure equitable partnership across academics, clinicians and survivors. Our Inclusive research strategy addresses additional expertise and resources for expanding the range of communities reached. Two additional charity leaders were consulted in developing this proposal along with a group of survivors from across the UK (n=5) on 4 occasions. These experts-by-experience strongly endorsed EDs as a focus for improvement; advocated for creative methods to represent survivors' complex health care experiences and dilemmas, and as tools to communicate and enhance practice; advised on practical areas such as separating stakeholder groups in Phase 1 workshops in preparation for joint co-design events. For Stage 2, we worked with the nominated Inclusion lead who reviewed the plain language summary, research plan and shaped the inclusion strategy. UHCW and CU funded PPI time (35 hours). We will regularly review effectiveness of the PPI approach and make changes accordingly including an end of project evaluation and feedback meeting with the partner. PPI reporting will follow GRIPP 2 recommendations.

3. Research team expertise

A multi-disciplinary team bringing leading expertise in sexual violence and trauma-informed care, emergency medicine, intervention design, implementation science and co-creation and strong partnerships with survivors, communities, service providers and global academic partners. The team has delivered over £1.5 million in NIHR-funded research, with demonstrable impact and expertise (see below).

Lead applicant LOD (10%): Lorna O'Doherty, Professor of Trauma, Mental Health & Recovery, Centre for Healthcare and Communities, Coventry University (CU) **Expertise:** Research in sexual violence and intersections with health and healthcare; development and evaluation of interventions for abuse Internationally recognised leader in sexual and domestic abuse research, co-investigator in RESTORE Centre of Research Excellence, a global collaboration aimed at transforming health systems to better respond to sexual violence. Leads an established research group at CU; 5-year grant capture of £4.2m, including chief investigator on grants valued at £1.8m; lead for NIHR-funded MESARCH study on health of survivors seeking care for sexual violence (2018-23), selected for reporting by NIHR Evidence and the BMJ and the basis of an Innovate UK Knowledge Transfer Partnership with NHS England (2025-27). Deep expertise in evidence synthesis, with 4 field-leading Cochrane Reviews. At forefront of novel ways to address survivors' needs using somatic/yoga interventions (MRC-funded Australia MANTRA trial) and arts and community (Home Office funded Upfront Survivors) where we use poetry written and performed by survivors to communicate findings e.g. Turn Up the Volume (youtube.com/watch?v=-gFcH840rOY). Co-developed a film with significant reach, to tackle abuse in young people's relationships. A background in health psychology and complex interventions, co-published a world-first trial of a complex primary care intervention for domestic violence in The Lancet which involved training 50+ GPs to respond more effectively to domestic abuse in women patients. Portfolio of funded research includes projects with minoritised populations/global majority e.g. multi-country study of migrant survivors' experiences (funded by European Commission).
Co-applicant CL (5%): Caroline Leech, Consultant in Emergency Medicine, UHCW; Honorary Associate Clinical Professor, University of Warwick **Expertise:** Emergency medicine; clinical translation and policy guidelines An experienced Consultant in Emergency Medicine with clinical experience of caring for patients who are survivors of sexual violence in the ED; led NHS quality improvements as Associate Medical Director and Major Trauma Clinical Lead at UHCW, Major Trauma Network Lead in the West Midlands, and committee chair for the Royal College of Emergency Medicine. CL has extensive experience in delivering advanced education and skills training for emergency department and pre-hospital care professionals. CL is the research lead for UHCW's ED, is a member of UHCW's Research Institute for Applied and Translation Technologies in Surgery and has been Principal Investigator for 8 successful NIHR Emergency Care portfolio studies.
PPI co-applicant AN (13%): Arti Naithani, Lived Experience Consultant, Survivors Voices **Expertise:** Public involvement and lived experience; arts-based methods A PPI expert representing our partner organisation, Survivors' Voices. Established in 2000, Survivors' Voices is survivor-led with expertise in enabling the involvement of survivors in research using trauma-informed and varied creative methodologies. AN brings expertise in advocacy and peer support, coordination of lived experience involvement and working with researchers and survivor-researchers. AN also has

experience in service improvement and campaigning for improved services for survivors, focusing particularly on marginalised groups. With background in drama and dance, AN has a passion for using creativity to enhance emotional/physical wellbeing bringing added strengths to this participatory arts-based research.
Co-applicant ELW (40%): Eleanor Lutman-White, PhD, joint Research Fellow, Coventry University and UHCW (40%) **Expertise:** Qualitative research methods and applications in vulnerable populations; survivor involvement in research A background in social work, and expertise in qualitative research with significant experience of working with vulnerable populations in research and on projects addressing health inequalities for patient groups across hospital and community settings. Has led qualitative research on children in care, smoking cessation in pregnancy, creative programmes for health, accessing diagnostic testing and implementing 'Martha's Rule' which has informed local and national policy. ELW has 5 years' experience of working with people affected by sexual and domestic/family abuse, interviewing hundreds of survivors as part of the NIHR funded MESARCH project with the PI. She has delivered training on trauma-informed care for NHS staff. Co-located at the Research Institute for Health Equity and Social Care at UHCW where she is responsible for advancing research for patient health and wellbeing.
Co-applicant NK (5%): Nikki Holliday, Design Manager, Research Methods and Evaluation Unit, Coventry University **Expertise:** Co-creation and intervention development NH brings extensive experience in applied health research, specialising in user-centred and co-created solutions. She has led and supported projects that connect NHS professionals, patients, carers, and the public through inclusive, iterative co-design methods. Her work bridges clinical expertise and lived experience to ensure research outputs are usable and meaningful across diverse health and wellbeing contexts, including dementia, frailty, chronic conditions, and caring e.g. led Innovate UK-funded 'Warm Neighbourhoods' project, which co-designed a telecare support service; UK PI Innovate UK-funded 'Develop in the Open' project, leading development of an electronic health record system. Her expertise spans service design, technology development, and implementation applied nationally and internationally. She is co-lead for Erasmus+ funded 'Co-Creating Wellbeing' project (international co-creation training); user-centred design advisor for the NIHR-funded RareCare project and member of the international PAAR-NET COST Association Working Group for Participatory Approaches with Older Adults. She has delivered training on co-creation to NHS staff and public and voluntary sector staff.

This team builds on several prior collaborations including the £1.3m NIHR-funded MESARCH project (2018-2023) led by LOD and on which ELW was part of a team that evaluated national service provision for sexual violence and conducted 1000 interviews with service providers, and survivors of sexual violence about post trauma stress and mental health, quality of life and experiences accessing services. ELW led on data collection, analysis and reporting of the qualitative research. PPI partner Survivors Voices was the team that led an independent in-depth evaluation of MESARCH's approach to working with people and communities. AN, as the named member of Survivors Voices and new to our research team, brings expertise in the arts to complement our planned outputs from the experience based co design work. We previously co-published learning from our work with survivors and communities with the proposed Inclusive research lead Roz Etwaria (RE). RE is a culture

transformation expert and routinely contributes PPI advisory work with LOD and ELW. RE was a guest speaker at the Sexual Violence and Health Research Network co-hosted by CU and UHCW in 2025. With a joint fellow position, ELW is embedded at UHCW where she works with CL on research projects in the emergency department. This partnership was the origin of the current proposal to build on our previous understanding of the negative (physical) health care journeys of survivors. CL brings essential links into clinical practice, emergency care, RCEM and reach to national networks. NH brings expertise in co-creation in the NHS and is also based at CU. Our clinical-academic partnership will benefit from the stability and commitment afforded by established relationships juxtaposed with the vigour of new interdisciplinary alliances. It will work to ensure the intervention has potential to be directly translatable to NHS settings whilst fulfilling expectations of survivors, service users, carers and patient advocacy groups on safety, acceptability and equity.

4. Inclusive research

Historically, victims and survivors of abuse have been omitted from discourses about them in mental health and healthcare, and across contexts, including research. When experiences of abuse intersect with racialised and minoritised identities, such as class, gender and disability, exclusion is even more pervasive. People with structural inequalities (e.g. poor living situations) are also more at risk of sexual/domestic abuse and exploitation. Several features enable us to reach and engage at risk, marginalised communities. Firstly, the context of delivery includes Coventry. With its cultural diversity and marked social and economic disadvantage, it is a 'Marmot city' and committed to tackling unfair differences in health. Warwick provides a contrast, with a predominantly White population which can result in challenges for representation and targeted service delivery for minority communities. CU leads on Coventry Health Determinants Research Collaboration and has a strong partnership with UHCW's Research Institute for Health Equity and Social Care, enabling diversification and reach. The participatory approach, minimal exclusion, and arts-based methods are measures counter to exclusion; they foster participation equity and elevate survivors' voices. We also offer options for aftercare and peer support via the PPI partner (covered in PPI budget) and recompense people's time through food vouchers.

The experiences of healthcare among individuals who experience structural barriers are central to advancing this research and the long-term benefit for all patients and communities. Considering the characteristics of our target sample, the NHS and the research questions we are asking, we have proposed a programme of inclusive research that is complementary but distinct from our PPI strategy. As such, we have sought a separate budget to support this activity. Firstly, to lead our inclusive research strategy, we invited one of our collaborators, Roz Etwaria (RE), a survivor-activist and leader in cultural transformation from an organisation that supports survivors from Black and People of Colour communities. RE applies a decolonisation and advocacy approach to challenge and dismantle the lingering effects of colonial power, systems of knowledge, and inequalities in institutions, relationships, and ways of thinking. RE's work on this project is towards epistemic justice ensuring experiential knowledge and voices are valued alongside academic knowledge. Their orientation emphasises healing, respect, and reciprocity in response to the intergenerational trauma and

disconnection because of colonisation. RE helped shape our proposal and would complement the PPI partner (with whom they already partner) in delivering the inclusive research strategy. They will train the team on involving survivors from Black and People of Colour communities in research, advise on embedding inclusion into the recruitment strategy and recruitment materials, support inclusion at workshops and co-design events and lead on inclusive knowledge mobilisation. We request additional budget, ringfenced for the following activities led by RE:

- Working locally with a funded diversity 'champion' and Coventry & Warwickshire Research Engagement Network community research champions to achieve 50% diversity in participants (neurodivergence, sensory, physical, learning disabilities, health, ethnicity, culture and language, gender minorities and older adults);
- Wide distribution of recruitment materials in multiple languages, and use of translation and interpreter services for a proportion of survivors (~5/30) to allow participation across all phases including translating subtitles of the short film output in the languages of participants as part of sharing accessible research outputs;
- Accommodate any participant requests to be interviewed by someone who reflects aspects of their identity by engaging with external expert interviewers;
- Fair distribution of resources to promote involvement – transport, accompanying carers, care costs, and taking steps to overcome digital poverty and barriers;
- Best practice in accessibility of spaces for different needs;
- Tackle barriers for NHS staff e.g. shift patterns, need for privacy and confidentiality.

****Knowledge mobilisation, dissemination and impact****

The long-term aim of this work is to see the widespread adoption of a trauma-sensitive, abuse-informed and survivor-centred model of care for the ED thereby making EDs safer for survivors and staff and addressing health inequalities for survivors of sexual violence (see Figure 1). This formative research will move us towards that aim through:

- Creative outputs including a short film reflecting survivors' experiences in the ED
- Co-produced theory of change and replicable intervention prototypes for evaluation
- 2 publications of discovery phase findings and co-design of intervention
- Grant application and added expertise (health economics, CTU) for feasibility study
- Early recommendations on meeting survivor and staff needs in emergency care.

To maximise the benefits of the current research and build networks, links and partnerships in preparation for Tier 2 research, we will deliver a proactive knowledge mobilisation strategy. This will be refined with PPI, clinical and NHS, and inclusion stakeholders throughout the project to ensure findings are framed in ways that are accessible and meaningful for varied audiences. Our approach balances breadth—engaging with multiple audiences such as survivors, underserved communities, patient advocacy groups, charities, the public—with depth, enabling focused collaboration with audiences such as NHS, sexual violence sector, commissioners, policymakers and institutions like the Royal College of Emergency Medicine (RCEM). We will use the opportunities to engage audiences as partners in interpreting, contextualising and planning for subsequent testing. Knowledge mobilisation will be ongoing and iterative, embedded across the lifecycle and use strategies that go beyond traditional dissemination, fostering dialogue, shared learning, and integrating feedback to inform theory of change and longer-term research and implementation.

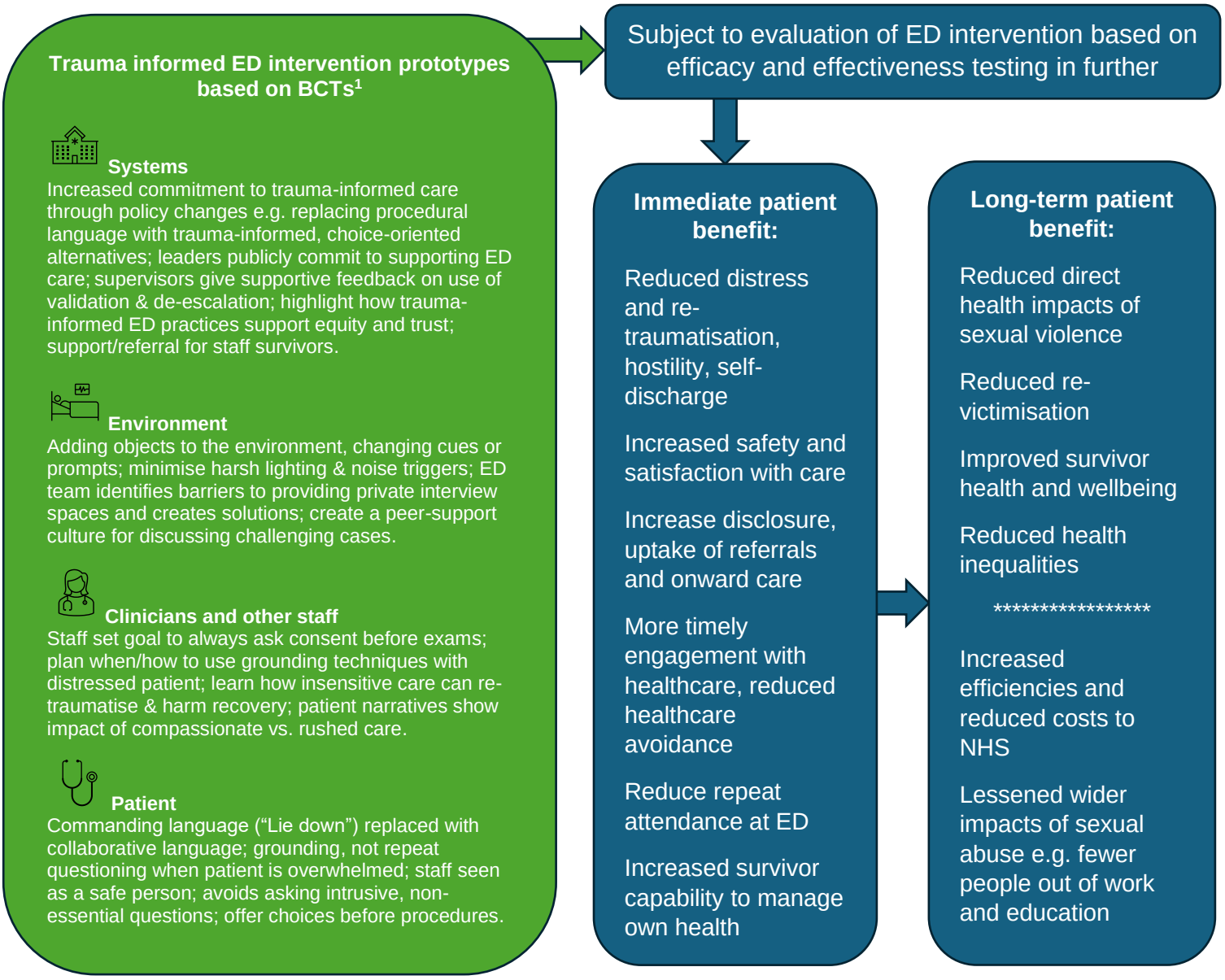

Figure 1 Route to patient benefit. As the project aims to identify stakeholder needs and map to intervention functions & content, this is a set of sample Behaviour Change Techniques (BCTs, from 93 available; 53) that could apply in intervention prototypes.

Impact will be achieved through:

Multi-format communication: We will co-develop a range of accessible outputs, including lay and professional summaries in written, audible and infographic formats, to ensure information reaches diverse audiences. All outputs will be co-created with survivor representatives to enhance clarity and resonance. We will use the project's creative outputs to communicate findings and facilitate engagement with survivor experiences of ED care. We will use social media and podcasts to stimulate conversation and host all project outputs on a project website.

Engagement with patients, the public and survivors of sexual trauma: Survivors Voices will lead our engagement with survivors and key organisations like Rape Crisis England and Wales; we will promote findings through public engagement events and collaborations with local community groups/voluntary sector partners and leaders.

Dissemination to researchers and practitioners: We will disseminate via our established national (e.g. Sexual Violence and Health Research Network) and international (e.g. RESTORE Centre of Excellence) partnerships and networks.
Clinical integration: We will publish in high-impact, peer-reviewed practice-oriented journals (e.g. Emergency Medicine Journal, Journal of Clinical Nursing); present at conferences (e.g. RCEM annual conference); and share findings with Trusts involved.
Policy influence: We will develop policy-relevant briefings, transferred via direct links with key professional organisations such as the RCEM (CL) and NHS England (LOD). The project will refine routes to influence practice with new evidence as it comes available e.g. update RCEM guidance on care for survivors of sexual abuse (see Appendix 3 RCEM Letter of Support) and shape NHS plans for ED mental health.
Sustained impact: We will monitor impact and adapt based on stakeholder feedback.

****Project management****
The project will be managed from the sponsor institution, Coventry University (CU), and under the auspices of the Abuse, Trauma and Health research group, led by LOD for the past 10 years. This group provides a resilient infrastructure and support, diverse staff and students, and ways of working that optimise the excellence and impacts of our research. The wider Centre offers access to methodological expertise within the Research Methods and Evaluation Unit and brings experience of supporting RfPB tier 1 and 2 studies hosted by UHCW. Within this context, ELW will be responsible for day-to-day project coordination, meeting weekly with LOD. With their co-location at the host institution UHCW, ELW will also liaise regularly with the safeguarding team and with CL on aspects that apply on site at UHCW and SWFT. The PPI and Inclusive research strategies will be run as sub-projects and feed into monthly meetings of the project team (ELW, CL, NH, AN with the inclusion team if funded) and chaired by LOD, where we will plan the phases of the EBCD, review progress and manage risks. Oversight will be provided by the sponsor institution, ensuring all governance requirements are met. The team has a strong track record in delivering NIHR-funded studies to time and target. UHCW will provide Research Management (5%) to support study set-up, and a Study Co-ordinator (5%). The project team will receive project manager support from Coventry's Research Management, Administration and Projects unit who will assist with administrative tasks, contracting and procurement, information governance protocols, risk-assessments and support the design and delivery of strategies to ameliorate risk and meet with LOD monthly to ensure project is running to timescales and budget. The project will be overseen by a study steering committee meeting twice annually, coinciding with funder reporting.

****Intellectual property and commercialisation****
No background IP will be used for the project. If any background IP is used, then this will be owned fully by the parties that brought the IP into the project. Foreground IP will be generated in the form of training resources and short film. Any foreground IP which partners are directly involved in developing are free to exploit outputs they developed directly. In any collaborative foreground IP generated, this will be jointly owned by the partners directly involved in creating it, unless contractually agreed otherwise. As part of the exploitation of the project is potential market adoption, commercial terms will be agreed with the sponsor and collaborative parties.

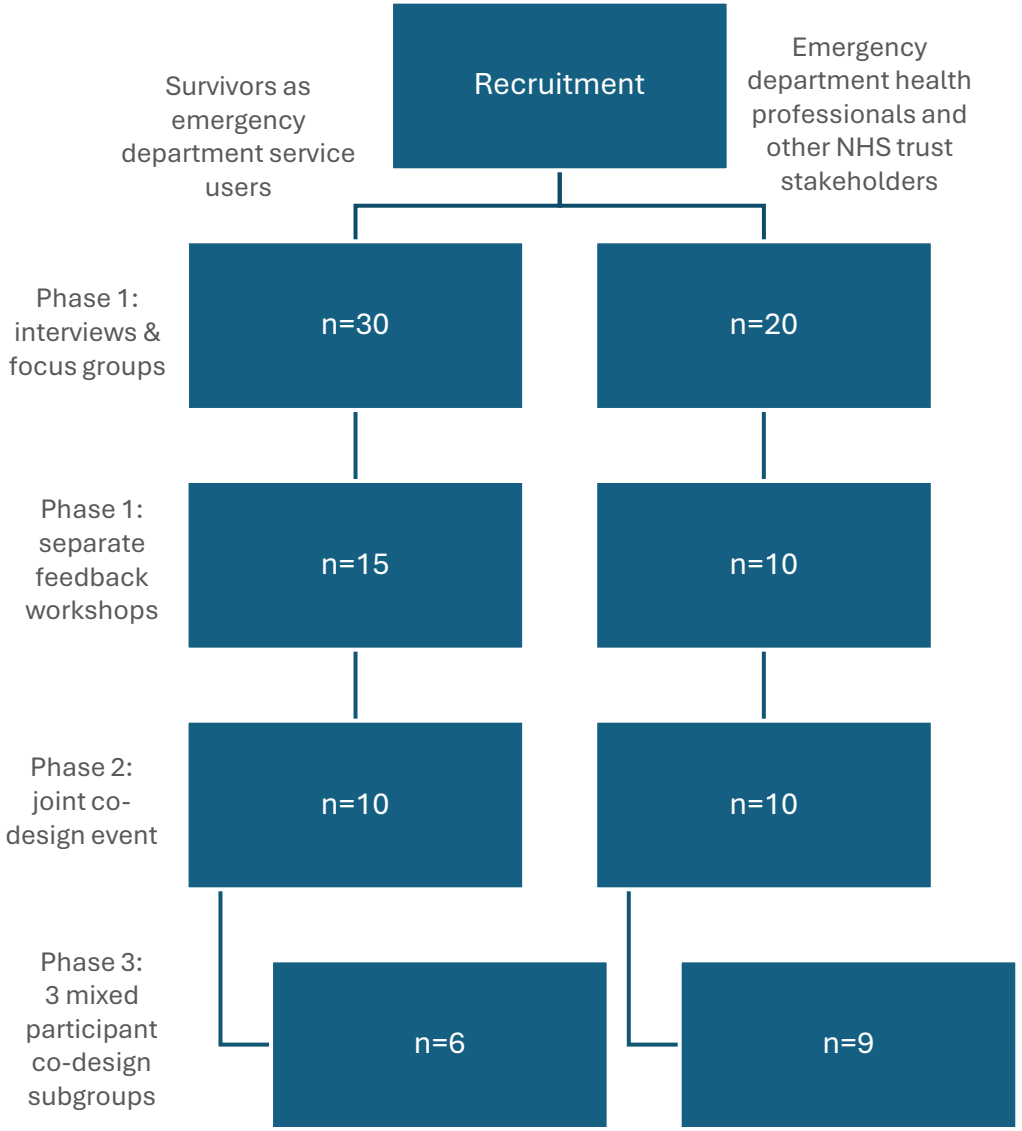

Figure 1 Flow of participants through Experience-Based Co-Design activities

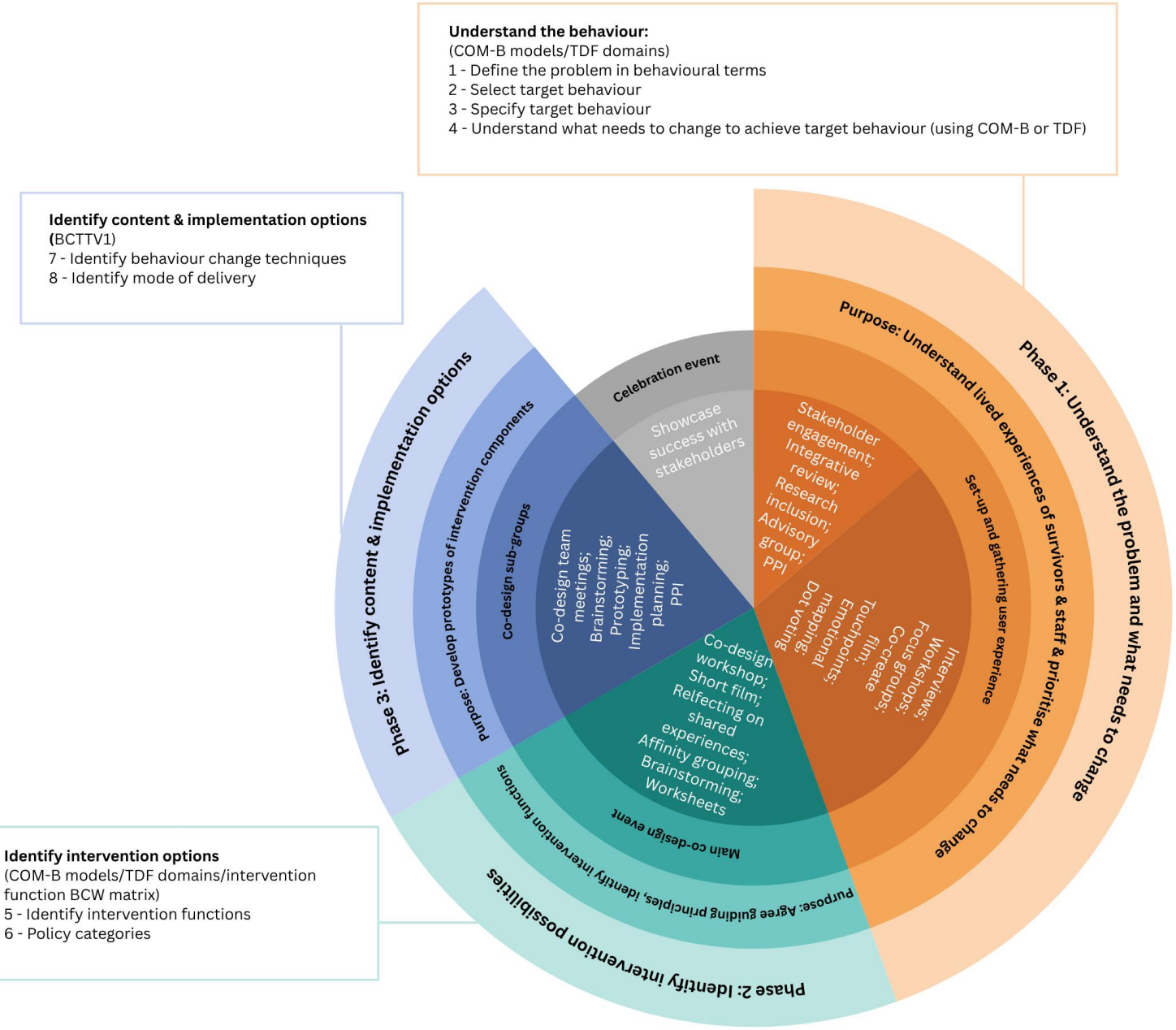

Figure 2 Integration of Experience-Based Co-Design and Behavioural Change Wheel

Calendar Year	**2026**									**2027**													**2028**					
	A	M	J	J	A	S	O	N	D	J	F	M	A	M	J	J	A	S	O	N	D	J	F	M	A	M	J
Month number	**Pre-study (~3mths)**			**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	**10**	**11**	**12**	**13**	**14**	**15**	**16**	**17**	**18**	**19**	**20**	**21**	**22**	**23**	**24**
Draft ethics applications																											
Co-design study materials																											
Create recruitment content																											
Website creation																											
Ethics applications + reviews																											
Study set-up at UHCW & SWFT																											
Inclusive research strategy*																											
Survivor recruitment & interviews																											
Recruit staff, focus groups																											
Qualitative data analysis																											
Develop creative outputs																											
Survivor workshop																											
Staff workshop																											
Co-develop activities for co-design event																											
Co-design event																											
Co-develop activities for co-design sub-groups																											
Co-design sub-groups																											
Develop theory of change																											
Develop prototypes																											
Funding bid for feasibility																											
Dissemination																											
Draft publications																											
Final report for funder																											
Research team meetings																											
SSC meetings																											

*Inclusive research strategy applied across several activities as per proposal

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Professor Lorna O'Doherty
Healthcare and Communities Transformation Research Centre
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6th November 2025

Dear Professor O'Doherty

Re: Enhancing inclusion and care of victim-survivors of lifetime sexual harm in the emergency department

As the Chair of the Royal College of Emergency Medicine Research Committee, I am writing to give my full support to this proposed study.

Sexual harm can have lifelong consequences for survivors, affecting not only their health but also their access to and experiences of healthcare. This study will generate valuable knowledge about the needs of patients who have experienced sexual harm and require emergency department care. It will also explore ways to enhance clinical practice so that care is fully trauma-informed and fosters patient engagement with healthcare services.

If funded, this study would provide important insights to inform RCEM guidance, for example, the 2015 guidance on care for victims of sexual abuse.

The RCEM Research Committee would be delighted to support this important study and will promote it directly via our clinical and research networks. We believe this study will add considerable value to the evidence base in this area and has the potential to improve patient care.

Kind regards

Matt Reed

Professor Matt Reed
Chair
Royal College of Emergency Medicine Research Committee

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Professor Lorna O'Doherty

Healthcare and Communities Transformation Research Centre

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CV1 5FB

Dear Lorna

Re: Enhancing inclusion and care of victim-survivors of lifetime sexual harm in the emergency department

I am writing to confirm South Warwickshire University NHS Foundation Trust fully supports an application to the NIHR Research for Patient Benefit scheme to fund the above-named 2-year project.

South Warwickshire University NHS Foundation Trust will champion the project locally and within its emergency department and facilitate staff recruitment to the focus groups and co-design activities within the project. Dr Miriam Anderson (Consultant in Emergency Medicine) will act as the local lead to enable these activities.

Sexual harm can have lifelong consequences for survivors, affecting not only their health but also their access to and experiences of healthcare. This study will generate valuable knowledge towards a novel model of care by exploring the needs of patients who have experienced sexual harm and require emergency department care. It will also examine staff experiences and ways to enhance clinical practice so that care is trauma-informed, fosters engagement with healthcare services and better outcomes for survivors of abuse in the long term.

Yours sincerely

J Williams

D Jones

Jo Williams

Head of Research and Development

Danielle Jones

Research Assistant Manager

crasac
Coventry Rape and Sexual Abuse Centre

11 November 2025

Reference: Re: An application to NIHR for RfPB funding for Enhancing inclusion and care of victim-survivors of lifetime sexual harm in the emergency department (NIHR507435)

To whom it may concern,

As the CEO of Coventry Rape and Sexual Abuse Centre (CRASAC), I am writing to give my full support to the proposed project.

We have supported Professor O'Doherty at Coventry University in their research efforts to improve outcomes for survivors over many years. We previously collaborated on the creation of a short film to raise awareness about domestic and sexual abuse in young people's relationships, which has had significant reach and impact. We supported Prof O'Doherty's groundbreaking NIHR research on sexual assault referral centres and health outcomes for survivors, referring our clients into the research, including survivors from culturally and linguistically diverse communities. We have spoken at several knowledge sharing events put on by the Coventry University team including being the provider of peer support services at a recent national event hosted at the University. We are aware this team conducts its research with great care, commitment and to the highest standard and prioritises the voices of survivors.

Sexual harm can have lifelong consequences for survivors, affecting not only a person's health but also their access to and experiences of healthcare. We are aware of the high levels of health inequalities experienced in communities locally and which significantly affect survivors and clients of CRASAC. There is an urgent need to support these individuals to access healthcare. This study will generate valuable knowledge about the specific experiences of patients who have experienced sexual harms and require emergency department care. It will also explore ways to enhance clinical practice so that care is fully trauma-informed and fosters patient engagement with healthcare services. We are keen to support this work by sharing about it with survivors accessing our advice, information and counselling services.

Yours sincerely,

Natalie Thompson

Chief Executive Officer
Natalie.Thompson@crasac.org.uk

14 November2025

Dear NIHR Funding Committee

Reference: Re: An application to NIHR for RfPB funding for Enhancing inclusion and care of victim-survivors of lifetime sexual harm in the emergency department (NIHR507435)

We write as specialist providers in trauma recovery, with first-hand insight into the challenges survivors of sexual harm face in urgent care settings. We have collaborated with Professor O'Doherty and the Coventry University team over the past few years, contributing to research, supporting outreach, and helping to widen the diversity of survivor voices represented in their studies. Their work is carried out with care, integrity, and deep respect for survivor experience. We offer our strong support for the project titled Enhancing inclusion and care of victim-survivors of lifetime sexual harm in the emergency department (NIHR507435).

This is an urgent safety issue. More than 10 million people in Britain are survivors of sexual offences, accounting for around 4 million A&E attendances each year. For many, the A&E environment is high-stress and often re-traumatising, leading some to avoid essential care and widening health inequalities.

Trauma responses are frequently misinterpreted as hostility or non-compliance. In a pressured A&E setting, this misunderstanding can escalate quickly, causing harm to patients and distress to staff.

This bid proposes a vital and practical solution. Through Experience-Based Co-Design, the project will develop an intervention that strengthens choice, consent, and safety within A&E—supporting stabilisation and dignity, even without disclosure.

This project represents a crucial investment. It promises more equitable care for millions of survivors and will help set a new benchmark for trauma-informed practice across the NHS.

We urge you to fund this essential research.

Yours faithfully,